AMERICAN PSYCHOLOGICAL ASSOCIATION

Journal of Counseling Psychology

© 2021 American Psychological Association
ISSN: 0022-0167

2021, Vol. 68, No. 6, 682–695
https://doi.org/10.1037/cou0000503

Patients' Individual Differences in Implicit and Explicit Expectations From the Therapist as a Function of Attachment Orientation

Sigal Zilcha-Mano[1], Tohar Dolev-Amit[1], Hadar Fisher[1], Tsachi Ein-Dor[2], and Bernhard Strauß[3]

[1] The Department of Psychology, University of Haifa

[2] Baruch Ivcher School of Psychology, Interdisciplinary Center (IDC) Herzliya

[3] Institute of Psychosocial Medicine and Psychotherapy, Friedrich-Schiller University Jena, University Hospital

Patients' attachment orientation was found to be an important predictor of the process and outcome of psychotherapy. The present study is the first to examine whether patients' attachment orientation toward significant others predicts their implicit and explicit expectations from the therapist, and whether this effect is moderated by the extent to which the therapist has become an attachment figure. In two studies ($N = 308$), we developed measures of implicit (lexical decision task) and explicit expectations from therapist, and tested the presence of individual differences in expectations as a function of the patients' attachment orientation, early and late in treatment. Whereas individuals higher on attachment anxiety did not report having fewer positive expectations from therapist early in treatment, they showed lower accessibility of positive expectations when measured implicitly. As treatment progressed, the extent to which the therapist has become an attachment figure may mitigate the adverse effects of pretreatment attachment anxiety.

Public Significance Statement

The article demonstrates how expectations from the therapist can be assessed using both explicit and implicit measures. Findings suggest that pretreatment attachment orientation can predict implicit and explicit expectations from the therapist, and suggest that potential adverse effects of insecure attachment can be mitigated when the therapist becomes an attachment figure.

Keywords: attachment, expectations, therapeutic relationship, implicit, lexical decision task

Supplemental materials: https://doi.org/10.1037/cou0000503.supp

This document is copyrighted by the American Psychological Association or one of its allied publishers.
This article is intended solely for the personal use of the individual user and is not to be disseminated broadly.
All rights, including for text and data mining, AI training, and similar technologies, are reserved.

It has been found that individuals' expectations affect their perceptions, motivations, behaviors, and emotions (e.g., Asch, 1946; Kelley, 1950; Secord, 1958), across many fields of science, including psychotherapy research (Constantino et al., 2018). Expectations from therapy are commonly defined as the patient's own prognostic belief about how personally helpful a given treatment in which they intend to engage, or have already begun to do so, will be (Constantino et al., 2018). Patient expectations are considered as a central mechanism of therapeutic change common to many, if not all types of psychotherapy (Frank, 1961; Goldfried, 1980; Rosenzweig, 1936) and pharmacological treatments (Zilcha-Mano et al., 2019). Patient expectations may refer to expectations regarding the outcome or the process of treatment, including expectations regarding the relationship with the therapist. Whereas expectations regarding the outcome of treatment have received much research attention and have been found to be related to outcome in a recent meta-analysis summarizing 81 independent samples (Constantino et al., 2018), expectations regarding the process of treatment remained relatively unresearched. Among all types of patient expectations regarding their treatment, one of the constructs receiving the least empirical attention is that of expectations regarding the relationship with the therapist. This gap in the literature is surprising, given that the most consistent predictor of treatment outcome across all types of psychotherapy is the working alliance between patient and therapist (Barber, 2009; Flückiger et al., 2018), and that theoretical conceptualizations view the alliance with the therapist, at least partly, as a function of the patient's internal representations (Bowlby, 1988; Fonagy, 2018) and expectations (Zilcha-Mano et al., 2014). The present study aims at moving a step closer to bridging this gap by investigating the potential attachment-based origins of the patient expectations from the therapist.

One of the most common approaches to study the therapeutic relationship is to focus on the working alliance between patient and

Editor's Note. Mark J. Hilsenroth served as the action editor for this article.—DMK Jr

This article was published Online First June 28, 2021.

Sigal Zilcha-Mano https://orcid.org/0000-0002-5645-4429

This work was supported by the Israeli Science Foundation (Grant no. 186/15).

Clinicaltrials.gov Identifier: NCT02728557 submitted on the 15.3.16.

None of the results reported in this manuscript have been published or are under consideration for publication elsewhere and none have been posted on recognized preprint servers. Some of the ideas and results presented in the manuscript were presented at the meeting of the Israel Society of Biological Psychiatry, Israel in March, 2018, and at the meeting of the Society of Psychotherapy Research, Amsterdam in June 2018.

Correspondence concerning this article should be addressed to Sigal Zilcha-Mano, Department of Psychology, University of Haifa, Mount Carmel, Haifa 31905, Israel. Email: sigalzil@gmail.com

This document is copyrighted by the American Psychological Association or one of its allied publishers.
This article is intended solely for the personal use of the individual user and is not to be disseminated broadly.
All rights, including for text and data mining, AI training, and similar technologies, are reserved.

therapist, a construct referring to the emotional bond that develops between them, and the degree to which they agree on the tasks and goals of treatment (Bordin, 1994). A stronger working alliance was found to be a significant predictor of subsequent treatment outcome, across psychotherapy orientations (Flückiger et al., 2018; Zilcha-Mano, 2017). Expectations regarding the working alliance have also been the subject of empirical research recently and have been defined as the patient's expectations before the start of the treatment of how strong the alliance with the therapist will be. As such, it is measured at baseline before the patient and therapist have any opportunity to interact (Barber et al., 2014). Higher expectations from the alliance with the therapist were found to predict stronger alliance during the course of treatment (Barber et al., 2014), lower risk of dropout from psychotherapy (Zilcha-Mano et al., 2016), and better treatment outcome (Barber et al., 2014). These findings suggest that expectations from the relationship with the therapist may have an important effect on the process and outcome of treatment (Pollak et al., 2008).

Although the literature sheds some light on the construct of expectations from the relationship with the therapist, much more is waiting to be discovered. First, the literature refers to expectations regarding the relationship with the therapist as a fixed entity, and measures it at only one time point, at baseline. Recent conceptualizations, however, reveal the dynamic nature of expectations (Constantino et al., 2018) and argue that they may change from one phase of treatment to the next. In the context of the relationship with the therapist, the baseline snapshot may capture more trait-like characteristics of the individual, whereas a snapshot taken further along the course of treatment may be a product of both the individual's trait-like characteristics and in-treatment processes that are a function of the developing relationship with the therapist. Second, the literature refers to only one dimension of expectations: how positive they are, that is, whether the expected alliance is strong enough. But negative expectations may have a profound effect as well. As it is the case for other constructs in psychology, negative expectations are not merely a synonym for low levels of positive expectations (Fredrickson, 2009). The literature on alliance expectations, however, does not disentangle positive versus negative expectations from the relationship with the therapists.

The effects of positive and negative expectations from the relationship with a significant other have been the focus of research from an attachment perspective. Over the past 40 years, attachment theory has emerged as one of the most important empirically grounded frameworks for understanding interpersonal relationships. According to attachment theory (Bowlby, 1973, 1980, 1982), people learn what they can expect from others in times of need based on a series of interactions with significant others, termed "attachment figures." These interactions, which begin to accumulate as early as infancy, are encoded in internal representations (called "internal working models") of the self and of others. Throughout one's life, these working models influence the individual's expectations, perceptions, and behaviors in interpersonal relationships. The measurement of attachment orientation can be broadly divided into two central traditions, which form distinct understandings of attachment rather than serve as a substitute for one another (Altmann et al., 2020; Crowell et al., 2008; Simpson et al., 2002; Waters et al., 2002). The first, stemming mostly from developmental traditions, measures representations of attachment, also known as internal working models, by assessing general and specific recollections from the individual's childhood, mainly through the use of interviews (e.g., the Adult Attachment Interview, AAI, George et al., 1996). The second, stemming largely from research on adult interpersonal relationships and social psychology, measures attachment orientation mostly by using self-report measures, such as the Experiences in Close Relationships (Brennan et al., 1998). Individual differences in adults' attachment working models are commonly conceptualized by the second tradition as creating two orthogonal dimensions of attachment orientation: attachment-anxiety and attachment-avoidance (Brennan et al., 1998). Individuals who score high on the anxiety dimension hold expectations of being rejected and disappointed in interpersonal relationships, and tend to adopt hyperactivating secondary attachment strategies: energetic attempts to attain greater proximity, support, and love as a means of regulating distress, combined with a lack of confidence that these will be provided. Preferring interpersonal closeness, anxiously attached individuals are expected to seek very close relationships with others. Those who score high on the avoidance dimension hold few positive expectations from others, and tend to adopt deactivating secondary attachment strategies: inhibition of proximity-seeking tendencies, maintenance of emotional and cognitive distance from others, potentially suppressing attachment-related thoughts and emotions as a means of regulating distress and reacting with compulsive self-reliance. People who score low on both dimensions are said to be securely attached and are thought to possess mental representations of comforting and support-providing attachment figures. Over a 100 studies have established that these two dimensions demonstrate different desires and expectations in interpersonal relationships (Fraley, 2019; Mikulincer & Shaver, 2016).

Studies investigating positive and negative expectations from attachment figures outside of the therapy room have demonstrated that distinct patterns of expectations are a product of the individual's attachment orientation. Baldwin et al. (1993) constructed a self-report scale to measure explicit expectations from attachment figures. They found individual differences in explicit expectations of the attachment figure for attachment-secure versus insecure individuals: both anxiously and avoidantly attached individuals showed lower levels of positive and higher levels of negative expectations. These findings rely exclusively on self-report measures, which is a clear limitation, given the potential effects of impression management and the ability to report on processes that may be colored by automatic processing. Over the years, the use of implicit measures in the study of individuals' differences as a function of attachment orientation reveals that such measures may add important new information (Petrowksi et al., 2019a; Spangler & Grossman, 1993). In fact, implicit and automatic measures yielded some of the most critical findings in attachment research over the years. For example, the use of physiological and social neuroscience measures sheds insight on what happens inside an avoidantly attached baby when the baby experiences separation and reacts as if not much has happened (Long et al., 2020; Zelenko et al., 2005). In adults, cognitive tasks have been found to be successful in measuring implicit and automatic process that are at least partly unaware, unintended, and uncontrollable, but at the same time affect behavior (Bargh, 1994; Banse & Kowalick, 2007; De Carli et al., 2016).

To examine expectations in a more implicit and less deliberate and controlled manner, Baldwin et al. (1993) also used a cognitive task, the lexical decision task. This task focused on the accessibility of positive and negative expectations, based on the assumption that

expectations that reflect the individuals' perceptions of others are more accessible (Beckes et al., 2017). Because of technical reasons, Baldwin et al. (1993) were able to test only the differences between securely and avoidantly attached individuals. They found that avoidantly attached individuals showed more susceptibility to negative expectations than did securely attached individuals. The findings of individual differences in explicit and implicit expectations as a function of attachment orientation were so robust that they were replicated even in relationships that are very different from those on which the working models were originally built, such as attachment to one's pet animal (Zilcha-Mano et al., 2011). These findings suggest that when focusing on explicit expectations from the attachment figure, both individuals higher on attachment anxiety and those higher on attachment avoidance report more negative expectations from the attachment figure. When focusing on explicit expectations from others, however, only individuals higher on attachment anxiety, and not those higher on attachment avoidance, show higher accessibility of negative expectations (Zilcha-Mano et al., 2011). Such contrast between implicit versus explicit expectations may indicate that whatever individuals report that they expect from others may differ from what is found when assessing such expectations in a less deliberate manner. This is especially important to consider in research fields that traditionally rely more on self-report measures.

The field of research on expectations from the therapist may gain from developments in the field of attachment research, and especially investigation of expectations from attachment figures. According to attachment theory, individual expectations from close relationships stem from early representations of the attachment figure. These expectations affect the individual's attitudes toward the partner in the relationship. In this way, these three constructs mutually affect one another and are involved in shaping and reshaping individual close relationships (Mikulincer & Shaver, 2016). Decades of empirical research suggest that patients' attachment orientations to significant others may affect the process and outcome of treatment (Levy & Johnson, 2019; Levy et al., 2018; Strauss, 2000; Strauss et al., 2006, 2017). Findings demonstrate an aggregated effect size of Cohen's $d = .35$ of attachment on outcome, albeit with much heterogeneity and with at least some of the effect being a product of the patients' pretreatment characteristics (for a meta-analysis, see Levy et al., 2018). Findings further suggest that therapists can function as an attachment figures for their patients (Mallinckrodt, 2010; Mallinckrodt, Gantt, et al., 1995; Mikulincer et al., 2013), with a potential for patients to earn attachment security (Bowlby, 1988; Kinley & Reyno, 2013; Strauß et al., 2018; Travis et al., 2001). Findings further suggest that the patients' general attachment orientation may affect their ability to form a secure attachment with the therapist: an individual with more secure attachment relationships outside the therapy room is more likely to form a strong alliance with the therapist (Bernecker et al., 2014; O'Connor et al., 2019; Strauss & Petrowski, 2017). These findings are consistent with theoretical conceptualizations and empirical findings on the potential role of representations of significant others in affecting the relationship with the therapist, such as object relationship conceptualizations (Klein, 1952; Winnicott, 1956), and the abundant literature on the Core Conflictual Relationship Theme (CCRT) approach (Barber et al., 2002; Crits-Christoph et al., 1990; Luborsky & Crits-Christoph, 1998) and on transference (Høglend et al., 2006, 2008, 2011). Indeed, it has been found that the patients' pretreatment representations of others substantially explained the alliance formed with the therapist during treatment (Zilcha-Mano et al., 2014). But to the best of our knowledge, the extent to which individuals' attachment outside of treatment can predict their positive and negative expectations from the therapist has not received any empirical attention to date.

The current study is a psychotherapy process research aimed at investigating (a) the extent to which the individuals' attachment orientation toward significant others can predict positive and negative expectations from the relationship with the therapist early (Session 1) and later (Session 12) in the course of treatment; and (b) the extent to which the therapist becoming an attachment figure can moderate the effect of pretreatment attachment orientation toward significant others on the negative and positive expectations from the therapist. To investigate these two aims, the present study adapted the existing measures for assessing expectations from attachment figures, as developed by Baldwin et al. (1993), to fit the relationship with the therapist. Following the literature on expectations from attachment figures, we used two measures: an explicit self-report scale and an implicit lexical decision task, to enable a full understanding of the construct of expectations, delving into both its explicit and explicit aspects (Mikulincer & Shaver, 2016). Petrowski et al. (2019b) have demonstrated the potential contribution of implicit measures to the study of the manifestations of attachment orientation in psychotherapy.

Given the absence of previous work on explicit versus implicit expectations from the therapist, it may be premature to formulate concrete hypotheses. It can generally be suggested that at least early on and at least at a declarative level (explicit expectations) patients may report expectations from their therapists that are congruent with those that they hold in their relationships with their attachment figures outside of the therapy room. Later, their expectations from their therapists may reflect not only such general trait-like tendencies, but also the attachment relationship that has formed with their therapist, potentially even representing a corrective experience, or earned security. On a more implicit level, the patients' levels of positive and negative expectations from the therapist have the potential to be affected by the specific setting in which the patient and therapist meet. In the current study, we examined our hypothesis during short-term treatment, which is becoming increasingly common. One of the defining characteristics of short-term treatments is the predetermined end of treatment. Such predetermined termination may raise attachment-related fears of abandonment in individuals who are high on attachment anxiety, and affect their sets of positive and negative expectations from their therapist. Thus, the investigation of positive and negative expectations from the therapist in short-term treatment among individuals who are high on attachment anxiety may be of special interest.

To achieve these aims, two studies were conducted: Study 1 focused on generating a self-report scale for measuring explicit expectations from the therapist, and investigating its associations with attachment orientation. Study 2 focused on generating a lexical decision task for measuring implicit expectations from the therapist. Study 2 measured implicit expectations from the therapist at two phases of treatment: early (Session 1) and late (Session 12) in the course of treatment, and examined the associations between the implicit expectations and general attachment orientation of the patient, as well as a potential moderating effect of the extent to which the therapist act as an attachment figure.

This document is copyrighted by the American Psychological Association or one of its allied publishers.
This article is intended solely for the personal use of the individual user and is not to be disseminated broadly.
All rights, including for text and data mining, AI training, and similar technologies, are reserved.

This document is copyrighted by the American Psychological Association or one of its allied publishers.
This article is intended solely for the personal use of the individual user and is not to be disseminated broadly.
All rights, including for text and data mining, AI training, and similar technologies, are reserved.

Study 1: The Development of the Therapist Expectations Scale (TES) for Measuring Explicit Expectations From the Therapist

Study 1 aimed at constructing an explicit measure of expectations from the therapist, the Therapist Expectations Scale (TES). Following Baldwin et al. (1993), we focused on patients' expectations regarding therapist behaviors in three attachment-relevant domains: dependence, trust, and closeness (see details below). These domains, chosen based on a theoretical formulation and substantial empirical research (for details, see Baldwin et al., 1993), were found to be applicable to diverse types of attachment figures (e.g., Zilcha-Mano et al., 2011), and were consistent with the accumulating literature on the role of the therapist as an attachment figure (e.g., Shaver & Mikulincer, 2002; Daly & Mallinckrodt, 2009, 2000). According to Baldwin's model, positive expectation from the other as an attachment figure involves the expectation that others will generally be "emotionally available and responsive and to interact in positive ways" (p. 746). Alternatively, negative expectations include the anticipation that the other will be "uniformly cold, rejecting, or manipulative" (p. 746). This model was extensively studied in the context of romantic relationships and the current study aims to adjust it to the context of patient relationships with the therapist. We evaluated the TES structure using factor analysis, and tested its reliability as well as its convergent validity against other related measures.

The convergent validity of the TES was tested against measures of attachment toward significant others in general, using the Experience in Close Relationships (ECR; Brennan et al., 1998) scale, against the alliance with the therapist, using the Working Alliance Inventory (WAI; Horvath & Greenberg, 1989), and against the Inventory of Interpersonal Problems (IIP; Alden et al., 1990; Horowitz et al., 1988). Based on the theoretical and empirical literature, we expected all three measures to be moderately associated with the TES because they capture related, albeit still distinct, constructs. Both the ECR and the TES are self-report questionnaires, based on attachment theory. Whereas the ECR items directly measure patient attachment orientation, the TES does not seek to replace existing scales measuring attachment orientation toward the therapist, but rather to measure patient expectations of a positive or negative response of the therapist when patients express wishes for dependence, closeness, and trust. Based on the theoretical literature, distinct patterns of expectation are assumed to underlie patients' attachment orientations. Previous studies have shown that people who score high on attachment anxiety or avoidance in close relationships are more likely to report more negative expectations and fewer positive expectations from their partners' behaviors (Baldwin et al., 1993). Regarding the alliance, both WAI and TES assess the relationship with the therapist. Whereas the WAI items measure the working alliance with the therapist, the bond between them, and agreement on the goals and tasks of treatment, the TES measures patient expectations of a positive and □ negative therapist response. Higher levels on the WAI are expected to be related to more positive and less negative expectations. Given that both the TES and the WAI focus on the same (patient-therapist) relationship, they are expected to be more strongly correlated than the TES with a measure that focuses on attachment relationships outside of the therapy room (Crowell et al., 2008; Mallinckrodt et al., 2005). The IIP measures behaviors linked with interpersonal problems outside of the relationship with the therapist. Previous studies have shown that people who score high on the IIP (indicating that they have more interpersonal problems), tend to form a poorer relationship with the therapist (Gibbons et al., 2003; Puschner et al., 2005). Higher levels on the IIP are expected to show an opposite pattern and be related to less positive and more negative expectations.

In sum, in accordance with the theoretical and empirical literature, we hypothesized that: (a) higher scores on the general attachment anxiety and avoidance scales are associated with more negative and less positive expectations from the therapist; (b) a stronger alliance is associated with more positive and less negative expectations from the therapist; and (c) more interpersonal problems of the patients are associated with more negative and less positive expectations from the therapist.

Method

Participants

We recruited 195 participants who were currently in therapy, or had been in therapy in the past, and asked them to complete the scales referring to their therapists. All participants were recruited using a snowball sampling method, through advertisements on social media, throughout the country and around the university, to assemble a diverse sample. All materials were completed online in Hebrew. Participants provided informed consent and received no compensation for participation. Participants were 154 women and 41 men; mean age was 28 (SD = 7.52); 179 of the participants were Jewish, six Muslim, five Christian, and five indicated "other;" 96 were single, 57 in a committed relationship, 40 married, and two divorced. Of the participants, 26.3% were in long-term dynamic psychotherapy, 10.9% were in short-term dynamic psychotherapy, 14.7% were in CBT, 1.3% were in integrative treatment, 3.2 were in couples or family therapy, and 44.2% did not know the type of treatment or would not share this information. Most of the participants were treated by clinical psychologists (75%), others by social workers (9.6%), psychiatrists (1.3%), and "others" (14.1%). Patients currently in therapy were in treatment for an average of 56.14 (SD = 82.30) weeks. No significant differences were found in any of the study variables between present or past therapy.

Measures

Experience in Close Relationships Scale (ECR; Brennan et al., 1998). The ECR is a 36-item self-reported measure assessing the construct of adult attachment. Participants rated the extent to which each item was descriptive of their experiences in close relationships on a 7-point Likert scale ranging from 1 (*not at all*) to 7 (*very much*). Eighteen items assessed attachment anxiety (e.g., "I worry about being abandoned") and 18 assessed attachment avoidance (e.g., "I prefer not to show a partner how I feel deep down"). The reliability and validity of these scales have been repeatedly demonstrated (Brennan et al., 1998; Mikulincer & Shaver, 2007). The Hebrew translation of the ECR has been used in hundreds of studies (Mikulincer & Shaver, 2016). In the present study, the two subscales of attachment anxiety and avoidance were used, with Cronbach's alpha of .92 for both.

Working Alliance—Short Form (WAI; Horvath & Greenberg, 1989). The WAI is a 12-item self-reported measure

assessing the alliance between the patient and therapist. Items were rated on a 7-point Likert scale, ranging from 1 (*never*) to 7 (*always*). In the present study, the total WAI score was used in all analyses, with Cronbach's alpha of .92.

Inventory of Interpersonal Problems—Circumplex—Short Form (IIP-C; Alden et al., 1990; Horowitz et al., 1988). The IIP is a 32-item self-reported inventory assessing behaviors linked with interpersonal problems. Individuals rate two types of items: interpersonal behaviors that are "hard for you to do" (e.g., "it is hard for me to let other people know when I am angry") and interpersonal behaviors that "you do too much" (e.g., "I am too afraid of other people"). Ratings of the degree to which each problem is distressing are made on a 5-point scale, ranging from 0 (*not at all*) to 4 (*extremely*). In the present study, the IIP total score was used in all analyses, with Cronbach's alpha of .90.

Procedure

The preparatory process for the study included the design of the TES, based on the measure of positive and negative expectations concerning a significant other's behaviors in the domains of dependence, closeness, and trust (Baldwin et al., 1993). The statements were built based on a pilot study preceding the present one, which included two stages: creation of a pool of potential items and selection of the most appropriate ones. In the first stage, we assembled a 50-item pool of "if . . . then . . . " statements relating to the dynamics of attachment to the therapist in the three domains: dependence, closeness, and trust. To generate the pool, 15 participants (all in past or present therapy, and not included in the sample of 195 patients who participated in testing the TES construct) were given general open-ended context question statements, adapted from Baldwin et al. (1993), and were asked to provide three positive and three negative sentence endings in each of the three domains (six sentences per participant): "If I need my therapist, my therapist will . . . " (dependence); "If I try and get close to my therapist, my therapist will . . . " (closeness); "If I trust my therapist, my therapist will . . . " (trust). From this pool of 90 items, the 50-item pool was generated based on the attachment theory emphasis for each domain. Consistent with Baldwin et al. (1993), all the sentence endings provided by the participants were reduced to one word (unless they were already supplied as a single word by the participants). This enabled us to use these words when examining the expectations from the therapists in the lexical decision task, a methodology borrowed from cognitive psychology.

In the second stage, we asked 20 participants who were at the time in therapy or had gone through therapy shortly before (not members of the previous samples) to rate the relevance of each item to the patient–therapist relationship, to indicate whether it described an experience they have had in their relationship with their therapist, to what extent, in their opinion, it may describe other people's experiences in their relationships with their therapist, and to specify whether each completion word was appropriate to the domain. For each item, participants were also asked to indicate whether it was clearly worded and understood, and whether the wording of any item in the database was vague, may be ambiguous, or misunderstood by potential responders. Finally, after answering all items, participants were briefly interviewed to assess how they understood each item, and to collect additional information about challenges in answering each item. Based on the answers of this sample, we made several small changes to refine and improve the wording of the questions. Additionally, we removed some of the items because they were said to be too obscure, because patients felt they were less relevant to the therapist, or because different participants understood them differently. At the end of this stage, 18 "if-then" items were found to be most suitable for assessing expectations from the relationship with the therapist. In each domain, the questions were arranged randomly. Participants were asked to read the 18 statements (six for each domain), think about their relationship with their therapist, and rate the extent to which each item described their feelings and thoughts in this relationship. Ratings were provided on a 7-point Likert scale, ranging from 1 (*not at all*) to 7 (*very much*).

A score was calculated for each of the three positive and three negative statements in each domain. Six items assess trust, three positive-related (e.g., "If I trust my therapist, my therapist will support") and three negative-related (e.g., "If I trust my therapist, my therapist will leave"). Six items assess closeness, three positive-related (e.g., "If I try and get close to my therapist, my therapist will accept") and three negative-related (e.g., "If I try and get close to my therapist, my therapist will withdraw"). Finally, six items assess dependence, three positive-related (e.g., "If I need my therapist, my therapist will help") and three negative-related (e.g., "If I need my therapist, my therapist will disappear"). The final version of the TES and the TES items' descriptive statistics are reported in the online supplements (Table S1).

Results and Discussion

Before exploring individual differences in the expectations from the therapist, we sought to examine whether it is necessary to refer to the positive and negative expectations in each of the three domains separately, or whether it is possible to create two homogeneous indexes that express positive and negative expectations from the therapist. Following previous studies that used similar methods (Baldwin et al., 1993; Zilcha-Mano et al., 2011), we expected to see a two-factor construct, one positive and one negative, which are moderately correlated.

Confirmatory Factor Analysis

To confirm the hypothesized two-factor structure of the TES, we conducted a Confirmatory Factor Analysis (CFA) using the MPlus 8.3 Structural Equation Modelling (SEM) package. Missing data were handled using Full-Information Maximum Likelihood (FIML). Model fit was estimated using Comparative Fit Index (CFI), Tucker Lewis Index (TLI), and Root Mean Square Error of Approximation (RMSEA). A model is deemed to be adequate if the CFI and TLI are higher than .95 and the RMSEA is lower than .07. The two-factor CFA model had excellent fit to the observed data, CFI = .97, TLI = .95, RMSEA = .06, $\chi^2(112) = 192.19$, $p < .05$, with factor loadings higher than .50 (see Table 1; average loading was .75 for positive expectations and .69 for negative expectations). A one-factor model did not have a good fit to the observed data.

The correlation matrix for the study variables is reported in the online supplements (Table S2). For positive expectations, high Pearson correlations were found between the average expectation items in each of the three domains, .62, .76, and .78, $p < .0001$, and the nine positive expectation items showed high reliability (Cronbach's alpha of .92). For the negative expectations, high Pearson

This document is copyrighted by the American Psychological Association or one of its allied publishers.
This article is intended solely for the personal use of the individual user and is not to be disseminated broadly.
All rights, including for text and data mining, AI training, and similar technologies, are reserved.

Table 1
Loading of Confirmatory Factor Analysis (Study 1)

Attachment-relevant domains	Positive	Negative
Positive Dependence 1	0.76	
Positive Closeness 1	0.68	
Positive Trust 1	0.75	
Positive Closeness 2	0.61	
Positive Closeness 3	0.64	
Positive Dependence 2	0.82	
Positive Trust 2	0.81	
Positive Dependence 3	0.81	
Positive Trust 3	0.85	
Negative Dependence 1		0.66
Negative Dependence 2		0.70
Negative Trust 1		0.73
Negative Trust 2		0.82
Negative Closeness 1		0.51
Negative Dependence 3		0.93
Negative Closeness 2		0.53
Negative Trust 3		0.75
Negative Closeness 3		0.57

correlations were found as well, .50, .50, and .80, $p < .0001$, and the nine negative expectation items revealed high reliability (Cronbach's alpha of .88). Thus, we created two single scores expressing explicit positive and negative expectations from the therapist, by averaging the nine items examining positive and negative expectations in the different domains, respectively. A significant negative correlation was found between positive and negative expectations, $r = -.40$, $p < .0001$. In all subsequent analyses, we used the two dimensions of positive and negative expectations.

To examine the unique contribution of attachment anxiety and avoidance (measured using the ECR) to explaining positive and negative expectations, we conducted a regression analysis. For positive expectations, the attachment dimensions explained 6% of the variance (R^2), $F(2, 312) = 11.30$, $p < .0001$. The regression coefficients show a significant unique contribution only for the avoidance dimension ($\beta = -.25$, $p < .0001$) and not for the anxiety dimension ($\beta = -.01$, $p = .85$), so that the higher participants' attachment avoidance, the lower their positive expectations from their therapist. For negative expectations, the attachment dimensions explained 8% of the variance, $F(2, 312) = 14.18$, $p < .0001$. Both avoidance ($\beta = .21$, $p < .0001$) and anxiety ($\beta = .13$, $p = .01$) show a significant unique contribution to explaining negative expectations, so that the higher participants' insecure attachment, the higher their negative expectations from their therapist. To assess the interactive effect of attachment anxiety and avoidance on expectations from the therapist, we tested the interaction between avoidant and anxious baseline attachment in predicting expectations from the therapist. For positive expectations, we found that the interaction was significant and explained .05% of the variance (R^2), $F(1, 74) = 4.49$, $p = .03$. For negative expectations, we found that the interaction was also significant and explained .06% of the variance (R^2), $F(1, 74) = 5.46$, $p = .02$.

At the next stage, we examined the associations between the TES on one hand and the WAI and IIP on the other. To this end, we calculated Pearson correlations to examine the association between positive and negative expectations and the WAI and IIP. As hypothesized, the higher the working alliance was, the higher the positive expectations ($r = .59$, $p < .0001$), and the lower the negative expectations from the therapist were ($r = -.43$, $p < .0001$). When tested against the three subscales of the WAI, for positive expectations, the WAI subscales explained 30% of the variance (R^2), $F(3, 311) = 45.05$, $p < .0001$, while for negative expectations, the attachment dimensions explained 18% of the variance, $F(3, 311) = 23.29$, $p < .0001$. Importantly, only the bond subscale remained significant for both positive (bond: $\beta = .54$, $p < .0001$; task: $\beta = .004$, $p = .96$; goals: $\beta = -.001$, $p = .99$) and negative expectation (bond: $\beta = .54$, $p < .0001$; task: $\beta = .004$, $p = .96$; goals: $\beta = -.001$, $p = .99$). In addition, as expected, the more interpersonal problems patients endorsed, the lower their positive expectations ($r = -.19$, $p < .0001$), and the higher their negative expectations were from their therapist ($r = .36$, $p < .0001$). All findings remained similar when repeating our analyses, controlling for the number of months in treatment (see online supplements).

Post Hoc Analyses

We examined the unique contributions of each of the significant predictors (attachment anxiety and avoidance, WAI, and IIP), in predicting both the positive and negative expectations. The analysis revealed that, for positive expectations, the regression coefficients show a significant unique contribution only for the alliance ($\beta = .48$, $p < .001$) and not for the avoidance dimension ($\beta = -.06$, $p = .32$), the anxiety dimension ($\beta = -.04$, $p = .47$), or interpersonal problems ($\beta = .04$, $p = .55$). For negative expectations, the regression coefficients show a significant unique contribution only for the alliance ($\beta = -.35$, $p < .001$) and not for the avoidance dimension ($\beta = .006$, $p = .92$), the anxiety dimension ($\beta = .05$, $p = .43$), or interpersonal problems ($\beta = .15$, $p = .052$). This can be explained by the fact that the general attachment orientation of the individual (as measured by the ECR) is a general attachment construct, rather than specific to the relationship with the therapist. The alliance, in contrast, is a construct that is specific to the relationship with the therapist.

Overall, the findings support the validity of the TES, and suggest that positive and negative expectations correlate with working alliance and interpersonal problems in meaningful ways, as expected. The findings also support our predictions that attachment anxiety and avoidance, as measured by the ECR, are associated with more negative and less positive expectations concerning therapist behavior. Specifically, higher levels of attachment anxiety were associated with higher levels of negative expectations, whereas higher levels of attachment avoidance were associated with both higher levels of negative expectations and lower levels of positive expectations. These findings are consistent with previous research regarding the unique patterns of expectations theoretically anticipated for patients with anxious versus avoidant attachment. Consistent with our hypothesis, the two measures used to assess the same relationship, the TES and the WAI, were more strongly correlated than a general measure of attachment orientation with significant others, although as the size of the correlation clearly shows, the two measures focusing on the relationship with the therapist, the WAI and the TES, are far from assessing the same construct. This pattern of correlations is consistent with previous research showing a strong correlation between attachment orientation toward the therapist and the alliance (.50–.80; Bachelor et al.,

This document is copyrighted by the American Psychological Association or one of its allied publishers.
This article is intended solely for the personal use of the individual user and is not to be disseminated broadly.
All rights, including for text and data mining, AI training, and similar technologies, are reserved.

2010; Mallinckrodt, Gantt, et al., 1995, 2005; Romano et al., 2008; Sauer et al., 2010). The finding that only the WAI bond subscale remained significant supports the theorized close relationship of working alliance and TES, while at the same time also pointing to their separateness[1]. Future studies should assess the three constructs (alliance, attachment, TES) repeatedly throughout the course of treatment, with all referring to the relationship with the therapist, to evaluate the levels of overlap among the three constructs. Such an inquiry would enable the exploration of the temporal relationships between them.

Although the findings of Study 1 are consistent with theoretical conceptualizations and past empirical findings, the assessment of patient expectations in Study 1 may still be challenged because of possible response biases and shared method variance of the self-report measures. The retrospective and correlational nature of Study 1 also pose several difficulties, such as the inability to assume causality. In addition, Study 1 did not explore the possibility that patient–therapist expectations may change over the course of a therapy. Study 2 was designed to address these shortcomings of Study 1. Finally, convergent validity was tested using the WAI and the conceptualization of Bordin for alliance. Future studies can benefit from testing the association with measures that also assess alliance from other perspectives.

Study 2: The Development of Lexical Decision Task for Measuring Implicit Expectations From the Therapist

For Study 2, we designed an implicit measure of expectations from the therapist, based on a lexical decision task (Meyer & Schvaneveldt, 1971), which is a well-known means of exploring a person's implicit working models (e.g., Baldwin et al., 1993; Beckes et al., 2017). In the lexical decision task, participants were presented with a string of letters on a computer screen and asked to indicate as quickly as possible whether the string formed an official word in Hebrew. In the present study, similar to Baldwin et al. (1993), participants read sentences that were either relevant or irrelevant to patient–therapist relations, in which the target words were located at the ends of the sentences. A target word, that is, the last string of letters in each sentence was either a positive therapist behavior word, a negative therapist behavior word, a neutral word, or a nonword (a meaningless string of letters). Participants were asked to decide whether the target stimulus was a word or not, and their response time was recorded. Reaction time (RT) served as the measure of the accessibility of these groups of words, so that the shorter the RT, the greater the accessibility (e.g., Fischler & Bloom, 1985). Overall, we had two sets of four word types: positive, negative, neutral, and nonwords; one set of each word type was relevant to patient–therapist relations, and one set irrelevant.

In accordance with the theoretical and empirical literature, we hypothesized that: (a) a contextual effect will be found, according to which participants will react faster to negative and positive therapist behavior words when they were placed in a meaningful context (i.e., patient-therapist relationships). This assumption was based on previous findings showing that processing the meaning of the sentence makes related words easier to retrieve from memory; (b) individual differences will be found in implicit positive and negative expectations from the therapist, early and late in treatment, as a function of the patients' general attachment orientation. We expected the contextual effect to be stronger when the target word matched participants' general attachment orientation at baseline: the higher a participant's attachment anxiety or avoidance, the shorter the participant's RT to negative therapist behavior words, and the longer the RT to positive therapist behavior words. Finally, (c) the effect of the patients' baseline general attachment orientation on expectations is moderated in later sessions of treatment by the extent to which the therapist acts as an attachment figure. Specifically, later in the course of treatment (Session 12), the attachment formed with the therapist is hypothesized to mitigate the adverse effects of insecure attachment.

Method

Participants

Participants in the study were enrolled in an ongoing RCT involving supportive-expressive therapy (SET) for depression, conducted at the University of Haifa (Trial registration: Clinicaltrials.gov Identifier: NCT02728557, study protocol: Zilcha-Mano et al., 2018). Patients were randomized to either supportive or supportive-expressive treatment for 16 weekly sessions at the University of Haifa Psychotherapy Research Lab. Seventy-six patients with MDD were enrolled in the present study, 45 women and 32 men. Mean age was 31.94 (SD = 8.89), and mean years of education 14.72 (SD = 2.8). Eighty-three percent were Jewish, 9% Christian, 5% Muslim, and 3% Atheist; 75.3% single, 18.2% married, and 6.5% divorced. All patients were fluent in Hebrew, and all study materials were in Hebrew. The same therapists provided both treatments. Seven therapists participated in the study; six women and one man, with a range of 6.5–21 years of clinical experience (M = 13, SD = 6.07). The mean number of patients each therapist treated was 10.85 (SD = 7.40).

Measures

Experience in Close Relationships Scale (ECR; Brennan et al., 1998). The ECR was used to assess patients' attachment anxiety and avoidance as in Study 1. Cronbach's alpha for the current study was .90 for anxiety and .89 for avoidance.

Therapist Expectations Scale (TES). The final version of the TES, as developed in Study 1, was used in Study 2 as well. Cronbach's alpha for the positive and negative expectations were .93 and .88, respectively.

Attachment Formation (ATF; Fraley & Davis, 1997). The ATF is a six-item self-reported measure assessing attachment-related functions. It is based on the three functions an attachment figure should fulfill based on attachment theory: proximity seeking, safe haven, and secure base. Items were rated on a 5-point Likert scale ranging from 1 (*Disagree*) to 5 (*Strongly agree*). We adapted this measure to examine the development of attachment formation to the therapist for the purpose of assessing the extent to which the therapist serves as an attachment figure. The adapted version of the

[1] For an elaborate discussion on the shared versus unique contributions of attachment and alliance and the justification for studying various components of the relationship between patient and therapist, please see the online supplemental material. For more information on disentangling expectations from the attachment relationship, the quality of the relationship, and attachment orientation in interpersonal relationships (Constantino et al., 2020).

This document is copyrighted by the American Psychological Association or one of its allied publishers.
This article is intended solely for the personal use of the individual user and is not to be disseminated broadly.
All rights, including for text and data mining, AI training, and similar technologies, are reserved.

ATF showed high reliability and validity (Ben David-Sela et al., 2020). Cronbach's alpha for the total score in the current study was .76

Designing the Lexical Decision Task for Measuring Implicit Expectations

In the preparatory process for the study, we formulated three therapist-relevant Hebrew sentences that were taken from the TES scale described above. They referred to the domains of trust ("If I trust my therapist, my therapist will . . . "), closeness ("If I try to get close to my therapist, my therapist will . . . "), and dependence ("If I need my therapist, my therapist will . . . "). We adjusted the stimuli to the therapist's gender. We also created three therapist-irrelevant Hebrew sentences that had the same grammatical structure and the same number of Hebrew words as the therapist-relevant sentences: "If the cab drives away, it will . . . " "If the new colorful checkered t-shirt is worn, it will . . . " and "If the old painting is hanging on the wall, it will . . . "

Immediately after the presentation of a context sentence, there was a short pause, after which one of 12 possible strings of Hebrew letters appeared on the screen. There were three Hebrew words reflecting positive expectations (e.g., "support"), three negative expectations (e.g., "disappoint"), and three neutral terms (e.g., "write"), and nine nonword letter sequences created by replacing one letter in one of the Hebrew words (e.g., "qrite" replacing "write"). Participants were asked to decide as quickly as possible whether a letter string was or was not a Hebrew word by pressing one of two possible keys on a computer keyboard. Word length and grammatical structure were the same across all categories.

The lexical decision task consisted of 108 trials, in each of which a sentence (therapist-relevant or irrelevant) ended with a positive expectation word, a negative expectation word, a neutral word, or a nonword. The task lasted 20 min on average. The 18 combinations of stimuli were presented six times, for a total of 108 trials. The trials were randomly ordered for each participant. The lexical decision task was conducted on a Pentium IBM-PC programmed with Superlab software. Brightness and contrast were set relatively low. The context sentences were displayed in pale blue lettering, and the target letter strings were displayed in red lettering on a black background in the middle of the monitor. Participants were working at their own pace. They first completed nine practice trials, followed by the 108 experimental trials. The context sentences and target letter strings in the practice trials were different from those in the experimental trials. Each trial began with the letter "X" displayed in the middle of the screen for 500 ms. Next, using a rapid serial presentation technique (Forster, 1970), the context sentences were displayed one word at a time on the screen, at a rate of 600 ms per word. After a 1,000 ms pause, the sentence was followed by one of the target stimuli. Participants were asked to judge as quickly as possible whether the target stimuli were a word or not by pressing the "Z" key for yes and the "/" key for no.[2] Following the key press, the target stimulus disappeared from the screen and the next trial began.

In the next stage, we asked 29 individuals who were not participating in the present trial, to take part in the lexical decision task. They were recruited at a university in Israel providing study credits for an undergraduate research methods course. At the time of the study, all participants were in therapy for a duration ranging from 2 to 84 months ($M = 23.62$, $SD = 24.79$). Overall, in this sample of 29 individuals, 4.1% of answers in the lexical task were wrong and were excluded from the analysis. For each participant, RTs for correct responses were averaged according to type of the target stimulus (neutral word, positive expectation word, negative expectation word, nonword) and context sentence (therapist-relevant, therapist-irrelevant). We conducted a two-way within-subject analysis of variance, with target stimulus and context sentence as the independent variables, and RT as the dependent variable. The analysis revealed a significant main effect for the context of the sentences, $F(1, 29) = 4.69$, $p = .03$, with RT for therapist-relevant sentences significantly shorter than for therapist-irrelevant sentences. The analysis also revealed a significant main effect for target stimulus, $F(3, 87) = 9.50$, $p < .0001$, with RT for positive expectation words being significantly shorter than for negative expectations, neutral words, and nonwords. The interaction effect was not significant. This pilot study supports the adapted version of the lexical decision task to assess accessibility of expectations from the relationship with the therapist, and show the expected contextual effect.

Procedure

The present study is part of a larger RCT (Zilcha-Mano et al., 2018). Potential patients were recruited by self-referral, based on advertisements. All participants provided written informed consent before joining the study and were informed that they had the right to withdraw from the research at any time. The procedures were approved by the Internal Review Board of the institution. Measures were administered at two pretreatment assessment meetings conducted by a research diagnostician, and subsequently on a session-by-session basis.

In the present study, we used the lexical decision task to assess implicit positive and negative expectations from the therapist at two time points: an early session (Session 1) and a late session in treatment (Session 12). To assess general attachment orientation, patients completed the ECR at baseline. To assess the extent to which the therapist acted as an attachment figure at Session 12, patients completed the ATF. The lexical decision task was administered before Sessions 1 and 12.

Results and Discussion

Preliminary Analyses

The estimated variances of the therapist random effect in the models were null and nonsignificant ($S^2 = 0.00$, $p = .99$, ICC = .00). We were interested first in examining whether we can replicate in Study 2 the patterns of associations found in Study 1 between the TES and ECR. To examine the unique contribution of baseline attachment avoidance and anxiety (measured by ECR) to explaining positive and negative expectations (measured by TES), we implemented a multilevel model, accounting for the hierarchical nature of the data set (repeated measures). For positive expectations, both attachment avoidance and anxiety were significant ($B = -.23$, $SE = .10$, $t(47) = -2.21$, $p = .03$; $B = .19$, $SE = .09$, $t(47) = 2.02$,

[2] These keys were chosen because they are the most convenient for a quick press on the Hebrew/English keyboards used in the current study.

This document is copyrighted by the American Psychological Association or one of its allied publishers.
This article is intended solely for the personal use of the individual user and is not to be disseminated broadly.
All rights, including for text and data mining, AI training, and similar technologies, are reserved.

$p = .04$, respectively). For negative expectations, both attachment avoidance and anxiety were significant ($B = .14$, $SE = .06$, $t(47) = 2.07$, $p = .04$; $B = .02$, $SE = .06$, $t(47) = 3.86$, $p = .0003$, respectively).

To test our prediction that general attachment orientations at baseline (ERC) are associated with implicit expectations regarding therapist behavior, we conducted multiple regression analyses examining the contribution of the two ECR scores (anxiety and avoidance) in predicting RTs for positive and negative behavior words in therapist-relevant sentences. We accounted for the general speed with which individuals react to negative words when calculating the RTs for responding to negative words in the context of the relationship with the therapist, and for the general speed with which individuals react to positive words when calculating the RTs for responding to positive words in the context of the relationship with the therapist. Thus, we calculated for each patient the relative RTs for positive and negative behavior words using the following formulas:

> Negative expectations were calculated with (t_ neutral—t_negative)/t_neutral
>
> Positive expectations were calculated with (t_ neutral—t_positive)/t_neutral

Higher scores, therefore, indicate *shorter* relative reaction times and thus *greater* accessibility to either positive or negative expectations in the relationship with the therapist. Analyses were repeated for an early session (Session 1) and for a late session (Session 12). For the late session, we also examined the moderating role of the extent to which the therapist acts as an attachment figure. To do so, we tested the interaction between baseline attachment orientation and the extent to which the therapist acts as an attachment figure in predicting positive and negative expectations from the therapist.

Session 1

Higher levels of attachment anxiety at baseline were significantly associated with longer RTs for positive expectations words in therapist-relevant sentences. This finding suggests that positive expectations from the therapist may be less accessible for patients higher on attachment anxiety. Avoidant attachment at baseline was not significantly associated with longer RTs for positive expectation words in therapist-relevant sentences. Anxious and avoidant attachment orientations at baseline were not significantly associated with RTs for negative expectation words in therapist-relevant sentences (Table 2).

Session 12

Higher levels of attachment anxiety at baseline were significantly associated with shorter RTs for negative expectations words in therapist-relevant sentences. This finding suggests that negative expectations from the therapist were more accessible for patients higher on attachment anxiety. Avoidant attachment at baseline was not significantly associated with RTs for negative expectation words in therapist-relevant sentences. Anxious and avoidant attachment orientation at baseline were not significantly associated with RTs for positive expectation words in therapist-relevant sentences (Table 2).

Table 2
Baseline Attachment Orientations and Positive and Negative Implicit Expectations From the Therapist

Session	Expectations	Attachment avoidance		Attachment anxiety	
		B	*p*	*B*	*p*
Session 1	Positive Expectations	−.01	.90	−.25	.03*
	Negative Expectations	−.11	.33	−.23	.052
Session 12	Positive Expectations	.07	.59	.23	.07
	Negative Expectations	.05	.67	.31	.01*

Note. * $p < 0.5$; The *B*s are standardized.

Examining the Moderating Effect of the Extent to Which the Therapist Acts as an Attachment Figure on the Effect of Baseline General Attachment Orientation on Expectations. In Session 12, we tested the interaction between baseline attachment orientation and the extent to which the therapist acts as an attachment figure in predicting positive and negative expectations from the therapist using the PROCESS macro of SPSS (Hayes, 2012). Predictors were mean-centered before the analysis. For negative expectations as the dependent variable, a significant interaction was found between baseline attachment anxiety and the extent to which the therapist served as an attachment figure ($B = -.08$, $SE = .04$, $t(55) = -2.01$, $p = .04$, $\Delta R^2 = 6.11\%$). To explore the meaning of the significant interaction, we conducted a bias-corrected and accelerated bootstrapping analysis, based on 5,000 repetitions (Hayes, 2017). Findings suggest that for patients who tended not to see the therapist as an attachment figure, a significant positive association was found between the baseline attachment anxiety levels and negative expectations ($B = -.10$, $SE = .03$, $t(55) = 2.86$, $p = .005$; $B = .05$, $SE = .02$, $t(55) = 2.10$, $p = .03$, for 1 *SD* lower than the mean level of ATF, and for the mean level of ATF, respectively). In contrast, for patients who tended to see the therapist as an attachment figure (1 *SD* higher than the mean level of the ATF), there was no significant relationship between the negative expectations and baseline attachment anxiety ($B = .007$, $SE = .03$, $t(55) = .21$, $p = .83$; see Figure 1). These findings suggest a potential protective effect of the attachment with the therapist, when formed.

For negative expectation as the dependent variable, no significant interaction was found between attachment avoidance and expectations ($B = -.08$, $SE = .07$, $t(55) = -1.14$, $p = .25$, $\Delta R^2 = 2.22\%$). Regarding positive expectations, no significant interaction was found for either attachment anxiety or attachment avoidance ($B = -.08$, $SE = .05$, $t(56) = -1.66$, $p = .10$, $\Delta R^2 = 4.40\%$; $B = -.01$, $SE = .09$, $t(56) = -.18$, $p = .85$, $\Delta R^2 = .06\%$, respectively).[3]

General Discussion

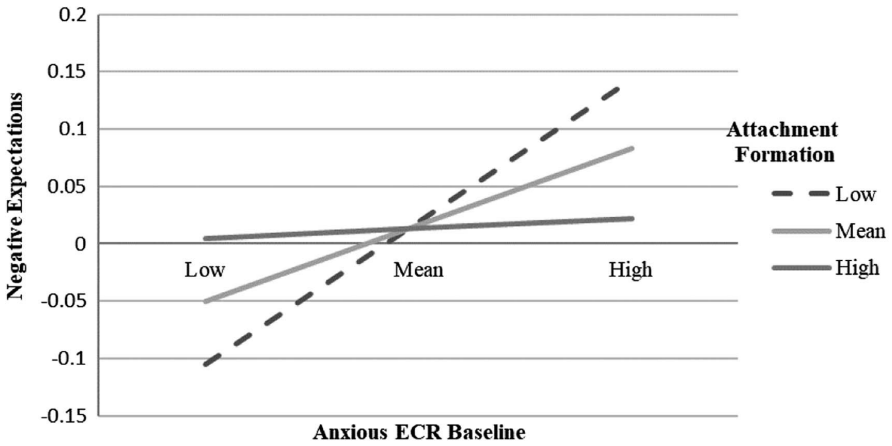
The present findings suggest that patients higher on anxious and avoidant attachment in their interpersonal relations show a distinct

[3] The association between the patient's baseline attachment anxiety and the extent to which the therapist served as an attachment figure was significant $r = .41$, $p = .001$. The association between baseline attachment avoidance and the extent to which the therapist served as an attachment figure was not significant $r = -.10$, $p = .43$.

This document is copyrighted by the American Psychological Association or one of its allied publishers.
This article is intended solely for the personal use of the individual user and is not to be disseminated broadly.
All rights, including for text and data mining, AI training, and similar technologies, are reserved.

Figure 1

The Moderating Effect of the Extent to Which the Therapist Acts as an Attachment Figure at Week 12 on the Effect of Baseline Attachment Anxiety on Implicit Negative Expectations From the Therapist



set of expectations from their therapists compared with more secure individuals. Taken together, insecure individuals tend to report more negative and less positive expectations from their therapist than more secure individuals. Importantly, different findings emerge when using explicit versus implicit methods to assess patients' expectations from their therapists. Whereas individuals higher on attachment anxiety do not explicitly report holding fewer positive expectations from the therapist early in treatment, they do show lower accessibility of positive expectations when measured implicitly. The differential pattern of results for explicit versus implicit measures is consistent with the literature demonstrating how essential it is to focus not merely on what an individual is able and interested in reporting but also on more automatic processing (Petrowski et al., 2019a, 2019b). Petrowski et al. (2019a) also showed that there is an incongruence between the Adult Attachment Interview (AAI), a gold standard measure of attachment, and the Implicit Association Test. The findings further suggest that as treatment progresses, the attachment formed with the therapist has the potential to mitigate the adverse effects of attachment anxiety.

Focusing on explicit expectations from the therapist, measured using a self-report questionnaire, patients with higher levels of avoidant attachment reported higher levels of negative and lower levels of positive expectations. This finding is consistent with the accumulating literature showing that individuals with higher levels of avoidant attachment tend to see less merit in being in close relationships with others (Mikulincer & Shaver, 2018). Patients with higher levels of attachment anxiety also reported higher levels of negative expectations, but they did not report lower levels of positive expectations. This is consistent with the literature suggesting that individuals with higher levels of attachment anxiety tend to adopt hyperactivating secondary attachment strategies. It is also consistent with the profile delineated in dozens of studies describing individuals with higher levels of attachment as seeing the merits of relationships with others, tending to idealize others (at least initially), and viewing them as capable of helping in a time of need, but at the same time also being insecure about the good intentions of others, and fearing being abandoned. Thus, they are preoccupied with the relationship and feel the need to cling on to it (Mikulincer & Shaver, 2018). The findings are also consistent with the literature suggesting that individuals with higher levels of attachment avoidance tend to adopt deactivating secondary attachment strategies and prefer self-reliance (Mikulincer & Shaver, 2016). The convergence of findings regarding explicit expectations from both studies, which are based on distinct designs (snowball vs. RCT) and populations (general treatment-recipients vs. patients with MDD enrolled in a clinical trial) supports the validity of these findings.

Focusing on implicit expectations, a more complex and intriguing pattern emerges. Significant results appear only for attachment anxiety. Specifically, at the beginning of the treatment, patients higher on attachment anxiety showed lower accessibility to positive expectations from the therapist. Later in the course of the treatment, however, patients higher on attachment anxiety showed higher accessibility to negative expectations from the therapist. A potential *post hoc* explanation is that early in the process of treatment, patients higher on attachment anxiety may try to protect themselves from assimilating their set of expectations to an individual who will soon leave them, in this case, the therapist in short-term treatment. The foretold termination of the relationship, which is one of the important characteristics of short-term treatment (Mann, 1973) may meet anxiously attached individuals exactly in the area of their fears of abandonment (Bowlby, 1988). In such instances, agreeing to form a relationship with a predefined ending may be associated with a strong motivation to protect oneself from becoming too deeply attached or even dependent.

As treatment progresses, however, it may be increasingly difficult for anxiously attached individuals not to project their potentially rigid negative expectations onto their relationship with the therapist. Indeed, at Session 12, patients with higher attachment anxiety in their interpersonal relationship showed higher levels of negative expectations from their therapists than more securely attached individuals. Importantly, the findings further suggest that such potentially automatic projection is mitigated by the therapist becoming an actual attachment figure for the patients. When the therapist

This document is copyrighted by the American Psychological Association or one of its allied publishers.
This article is intended solely for the personal use of the individual user and is not to be disseminated broadly.
All rights, including for text and data mining, AI training, and similar technologies, are reserved.

becomes an attachment figure, the insecure negative expectations that generally characterize these patients in their relationships outside of treatment may not be automatically projected onto the therapist. These findings may, in some ways, resemble the notion of earned security, according to which insecurely attached individuals may be able to form more secure representations of an attachment figure when the new attachment figure is different (more reliable, less malevolent) from those the insecure individuals may currently have in their lives (Hesse, 2008; Saunders et al., 2011). In such cases of earned security, the relationship with the therapist may serve as a corrective interpersonal experience for the patient (Castonguay & Hill, 2012). According to attachment theory, a single highly supportive figure has the potential to have a tremendous effect on the individual (Bowlby, 1973).

It is possible to speculate *post hoc* on the reasons why significant associations between attachment avoidance and positive and negative expectations emerged only for explicit expectations but not for implicit ones. Individuals with higher levels of attachment avoidance may report forming expectations from their therapists that are similar to those that they have from their significant others. But at a more subconscious level, it may appear that they are less inclined to see the therapist as a meaningful other to whom they assimilate their set of expectations. This *post hoc* speculation should be more directly tested in future studies that compare explicit attachment to significant others and to the therapist for each individual. Other complementary potential *post hoc* explanations may refer to the theorized overreliance of individuals with higher levels of attachment avoidance on repressive-dissociative mechanisms, their denial of attachment needs, and their focus on "compulsive" attempts to attain self-reliance as a way of compensating for the reluctance to depend on others. At the same time, they may have less access to internal representations of attachment figures (Edelstein & Shaver, 2004; Mikulincer, 1998).

If replicated in future studies, the current findings may have important clinical implications. If our earned security hypothesis receives direct support from future studies, it has the potential to inspire future mental health interventions. The findings carry a message of warning together with one of hope. On the one hand, the findings demonstrate the deterministic side of attachment theory: individuals are likely to automatically project their attachment-based expectations even onto the relationship with the therapist. The same maladaptive expectations that characterize anxiously attached individuals outside of the therapy room may be automatically projected onto their relationship with the therapists. Even when facing a benevolent figure who may not seek to disappoint one and may not leave and abandon one, the individual may still project insecure attachment expectations onto that figure. But on the other hand, the findings also suggest *when* an individual is likely to be able to form more adaptive expectations from the therapist: when the therapist actually becomes an attachment figure. Thus, forming a real attachment relationship with the patient (Gelso et al., 2013) may protect anxiously attached individuals from automatically projecting their maladaptive expectations from others onto the relationship with the therapist. One way in which the therapist may be able to achieve that is by gradually contrasting the patients' maladaptive expectations (Daly & Mallinckrodt, 2009). As stressed by one of the therapists interviewed in Daly and Mallinckrodt (2009) study: "I reassure that I'm not going to judge them or reject them. And if I know specifically how they're afraid of being hurt, and I would hope to establish that and any antecedents, such as people being unkind, estrangement, abandonment, then I probably would try to include in the therapy, either subtly or very deliberately, indications that I'm going to be available and I am not going to hurt them in that way" (p. 558).

An important limitation of the present study is the flip side of one of its main merits: adapting implicit and explicit measures to expectations from the therapist. Using a series of studies with a total of 308 participants, we adapted this measure from general interpersonal relationships to the relationship with the therapist. Although these measures have been successfully adapted before to other types of relationships (Zilcha-Mano et al., 2011), and underwent rigorous pilot testing in the current series of studies, their psychometric properties are yet to be fully established in future studies. For example, the high correlation between explicit TES and the measure of alliance raises a question regarding its unique contribution. However, when the design enables patients to clearly contextualize their response and refers to a specific point in time (baseline, Session 12), there is a clear differentiation between alliance and attachment in predicting the TES, as evident in the findings of Study 2. It could be that when the patients are asked to answer about their treatment without a clear contextual limit of the time frame which they need to refer to, it is harder to differentiate the two, as evident in the findings of Study 1. Future studies should continue and study this possibility to enable reliable assessment of the patients' expectations.

Given the universal aspects of attachment and its potential culture-specific manifestations (Grossmann et al., 2005), the present findings need to be replicated with other populations. Other limitations include the use of the snowball sampling method in Study 1, a nonprobability method in which random sampling cannot be assumed (e.g., Magnani et al., 2005), and the fact that in Study 1, the time that had elapsed since the end of treatment was not collected for patients in past therapy. Additionally, although in Study 2 attachment orientation was assessed pretreatment, before the other measures were collected, causality cannot be assumed or determined because patient expectations were not manipulated.

Given that any cognitive task focusing on implicit processing has its own advantages and disadvantages, future studies should examine the replicability of the current findings using a variety of additional tasks, such as the Implicit Association Test (IAT; Greenwald et al., 1998; Petrowski et al., 2019a, 2019b) and the Affect Misattribution Procedure (AMP; Payne et al., 2005). In addition, the implicit association test (IAT) is sensitive to both language and cultural differences (Greenwald et al., 1998; Ogunnaike et al., 2010). Therefore, to be able to generalize the results, future studies should replicate the current observations with patients from different cultures and speaking other languages.

Future studies should further explore the roles of patients' expectations from the therapists, seeking to determine for whom expectations may play a greater role (moderation models) and how (mediation models). Interpersonal research outside of psychotherapy suggests that both explicit and implicit measures contribute unique variance to predicting the individual's well-being (Banse et al., 2013), therefore future studies should examine how explicit and implicit positive and negative expectations from the therapist may affect subsequent treatment outcome, referring to a diversity of outcome constructs. Given the designs of the present studies

This document is copyrighted by the American Psychological Association or one of its allied publishers.
This article is intended solely for the personal use of the individual user and is not to be disseminated broadly.
All rights, including for text and data mining, AI training, and similar technologies, are reserved.

(Study 1, as a single snapshot design, and Study 2, as an ongoing trial), we were not able to examine potential associations with treatment outcomes. The potential interactive effect of alliance and attachment should also receive attention in future investigations, further clarifying the potential unique and common contribution of each (Mallinckrodt, Gantt, et al., 1995). Finally, the current findings are based on a self-report measure of attachment. Future studies should examine the extent to which the implicit versus explicit expectations from the therapist are related to the patients' scores on the Adult Attachment Interview (George et al., 1996) versus self-report measures of attachment.

The present study integrates psychotherapy research with methods and theoretical conceptualizations derived from cognitive psychology and attachment theory. The current findings demonstrate the merit of assessing both implicit and explicit expectations that individuals have from their therapists. Given the importance of the relationship with the therapist for treatment success (Flückiger et al., 2018), understanding the expectations one has from the relationship with the therapist can be instrumental in developing personalized treatment adapted to one's attachment orientations. In this way, the accumulating evidence from hundreds of studies on attachment (Cassidy & Shaver, 2016) can be put to use to guide the effective delivery of patient-tailored clinical practice.

References

Alden, L. E., Wiggins, J. S., & Pincus, A. L. (1990). Construction of circumplex scales for the Inventory of Interpersonal Problems. *Journal of Personality Assessment*, *55*, 521–536.

Altmann, U., Nodop, S., Dinger, U., Ehrenthal, J. C., Schauenburg, H., Dymel, W., Willutzki, U., & Strauss, B. M. (2020). Differential effects of adult attachment in cognitive-behavioral and psychodynamic therapy in social anxiety disorder: A comparison between a self-rating and an observer rating. *Clinical Psychology & Psychotherapy*.

Asch, S. E. (1946). Forming impressions of personality. *The Journal of Abnormal and Social Psychology*, *41*, 258–290.

Bachelor, A., Meunier, G., Laverdiére, O., & Gamache, D. (2010). Client attachment to therapist: Relation to client personality and symptomatology, and their contributions to the therapeutic alliance. *Psychotherapy: Theory, Research, Practice, Training*, *47*, 454.

Baldwin, M. W., Fehr, B., Keedian, E., Seidel, M., & Thomson, D. W. (1993). An exploration of the relational schemata underlying attachment styles: Self-report and lexical decision approaches. *Personality and Social Psychology Bulletin*, *19*, 746–754.

Banse, R., Imhoff, R., Steffens, M., Schramm, N., Rösch, A., Roberts, M., & Stangier, U. (2013). Partner-AMP and well-being: Evidence for an implicit secure base script? *Personal Relationships*, *20*, 140–154.

Banse, R., & Kowalick, C. (2007). Implicit attitudes towards romantic partners predict well-being in stressful life conditions: Evidence from the antenatal maternity ward. *International Journal of Psychology*, *42*, 149–157.

Barber, J. P. (2009). Toward a working through of some core conflicts in psychotherapy research. *Psychotherapy Research*, *19*, 1–12.

Barber, J. P., Foltz, C., DeRubeis, R. J., & Landis, J. R. (2002). Rigidity of interpersonal themes in narratives about relationships. *Psychotherapy Research*, *12*, 139–158.

Barber, J. P., Zilcha-Mano, S., Gallop, R., Barrett, M., McCarthy, K. S., & Dinger, U. (2014). The associations among improvement and alliance expectations, alliance during treatment, and treatment outcome for major depressive disorder. *Psychotherapy Research*, *24*, 257–268.

Bargh, J. A. (1994). The four horsemen of automaticity: Awareness, intention, efficiency, and control in social cognition. *Handbook of Social Cognition*, *1*, 1–40.

Beckes, L., Simons, K., Lewis, D., Le, A., & Edwards, W. (2017). Desperately seeking support: Negative reinforcement schedules in the formation of adult attachment associations. *Social Psychological and Personality Science*, *8*, 229–238.

Ben David-Sela, T., Dolev-Amit, T., Eubanks, C. F., & Zilcha-Mano, S. (2020). Achieving successful resolution of alliance ruptures: for whom and when? *Psychotherapy Research*, 1–12. https://doi.org/10.1080/10503307.2020.1862432

Bernecker, S. L., Levy, K. N., & Ellison, W. D. (2014). A meta-analysis of the relation between patient adult attachment style and the working alliance. *Psychotherapy Research*, *24*, 12–24.

Bordin, E. S. (1994). Theory and research on the therapeutic working alliance: New directions. In A. O. Horvath & L. S. Greenberg (Eds.), *The Working Alliance: Theory, Research, and Practice* (pp. 13–37). Wiley.

Bowlby, J. (1973). Attachment and loss: Vol. 2. *Separation: Anxiety and anger*. Basic Books.

Bowlby, J. (1980). *Attachment and loss: Sadness and depression*. Basic Books.

Bowlby, J. (1982). Attachment and loss: Vol. 1. *Attachment* (2nd ed.). Basic Books.

Bowlby, J. (1988). *A secure base: Clinical applications of attachment theory*. Routledge.

Brennan, K. A., Clark, C. L., & Shaver, P. R. (1998). Self-report measurement of adult romantic attachment: an integrative overview. In R. W. S. Simpson (Ed.), *Attachment theory and close relationships* (pp. 46–76). Guilford Press.

Cassidy, J., & Shaver, P. R. (2016). *Handbook of attachment: Theory, research, and clinical applications* (3rd ed.). Guilford Press.

Castonguay, L. G., & Hill, C. E. (2012). *Transformation in psychotherapy: Corrective experiences across cognitive behavioral, humanistic, and psychodynamic approaches*. American Psychological Association.

Constantino, M. J., Coyne, A. E., Goodwin, B. J., Vîslă, A., Flückiger, C., Muir, H. J., & Gaines, A. N. (2020). Indirect effect of patient outcome expectation on improvement through alliance quality: A meta-analysis. *Psychotherapy Research*, 1–15.

Constantino, M. J., Vîslă, A., Coyne, A. E., & Boswell, J. F. (2018). A meta-analysis of the association between patients' early treatment outcome expectation and their posttreatment outcomes. *Psychotherapy*, *55*, 473–485.

Crits-Christoph, P., Demorest, A., & Connolly, M. B. (1990). Quantitative assessment of interpersonal themes over the course of psychotherapy. *Psychotherapy: Theory, Research, Practice, Training*, *27*, 513–521.

Crowell, J., Fraley, R. C., & Shaver, P. R. (2008). Measures of individual differences in adolescent and adult attachment. In J. Cassidy & P. R. Shaver (Eds.), *Handbook of attachment: Theory, research, and clinical applications* (2nd ed., pp. 599–634). Guilford Press.

Daly, K. D., & Mallinckrodt, B. (2009). Experienced therapists' approach to psychotherapy for adults with attachment avoidance or attachment anxiety. *Journal of Counseling Psychology*, *56*, 549–563.

De Carli, P., Tagini, A., Sarracino, D., Santona, A., & Parolin, L. (2016). Implicit attitude toward caregiving: The moderating role of adult attachment styles. *Frontiers in Psychology*, *6*, 1906.

Edelstein, R. S., & Shaver, P. R. (2004). Avoidant attachment: Exploration of an oxymoron. *Handbook of Closeness and Intimacy*, *39*, 7–412.

Fischler, I. S., & Bloom, P. A. (1985). Effects of constraint and validity of sentence contexts on lexical decisions. *Memory & Cognition*, *13*, 128–139.

Flückiger, C., Del Re, A. C., Wampold, B. E., & Horvath, A. O. (2018). The alliance in adult psychotherapy: A meta-analytic synthesis. *Psychotherapy*, *55*, 316–340.

This document is copyrighted by the American Psychological Association or one of its allied publishers.
This article is intended solely for the personal use of the individual user and is not to be disseminated broadly.
All rights, including for text and data mining, AI training, and similar technologies, are reserved.

Fonagy, P. (2018). *Attachment theory and psychoanalysis*. Routledge.
Forster, K. I. (1970). Visual perception of rapidly presented word sequences of varying complexity. *Perception & Psychophysics*, *8*, 215–221. https://doi.org/10.3758/BF03210208
Fraley, R. C. (2019). Attachment in adulthood: Recent developments, emerging debates, and future directions. *Annual Review of Psychology*, *70*, 401–422.
Fraley, R. C., & Davis, K. E. (1997). Attachment formation and transfer in young adults' close friendships and romantic relationships. *Personal Relationships*, *4*, 131–144.
Frank, J. D. (1961). *Persuasion and healing: A comparative study of psychotherapy*. The Johns Hopkins Press.
Fredrickson, B. (2009). *Positivity*. Harmony.
Gelso, C. J., Palma, B., & Bhatia, A. (2013). Attachment theory as a guide to understanding and working with transference and the real relationship in psychotherapy. *Journal of Clinical Psychology*, *69*, 1160–1171.
George, C., Kaplan, N., & Main, M. (1996). *Adult attachment interview*. [Unpublished manuscript]. University of California, Berkeley.
Gibbons, M. C., Crits-Christoph, P., de la Cruz, C., Barber, J. P., Siqueland, L., & Gladis, M. (2003). Pretreatment expectations, interpersonal functioning, and symptoms in the prediction of the therapeutic alliance across supportive-expressive psychotherapy and cognitive therapy. *Psychotherapy Research*, *13*, 59–76.
Goldfried, M. R. (1980). Toward the delineation of therapeutic change principles. *American Psychologist*, *35*, 991–999.
Greenwald, A. G., McGhee, D. E., & Schwartz, J. L. K. (1998). Measuring individual differences in implicit cognition: The implicit association test. *Journal of Personality and Social Psychology*, *74*, 1464–1480.
Grossmann, K. E., Grossmann, K., & Keppler, A. (2005). Universal and culture-specific aspects of human behavior: The case of attachment. In W. Friedlmeier, P. Chakkarath, & B. Schwarz (Eds.), *Culture and human development: The importance of cross-cultural research for the social sciences* (p. 75–97). Psychology Press/Erlbaum (UK) Taylor & Francis.
Hayes, A. F. (2012). *PROCESS: A versatile computational tool for observed variable mediation, moderation, and conditional process modeling* [White paper]. http://www.afhayes.com/public/process2012.pdf
Hayes, A. F. (2017). *Introduction to mediation, moderation, and conditional process analysis: A regression-based approach*. Guilford Publications.
Hesse, E. (2008). The Adult Attachment Interview: Protocol, method of analysis, and empirical studies. In J. Cassidy & P. R. Shaver (Eds.), *The handbook of attachment: Theory, research and clinical applications* (pp. 552–598). Guilford Press.
Høglend, P., Amlo, S., Marble, A., Bøgwald, K. P., Sørbye, Ø., Sjaastad, M. C., & Heyerdahl, O. (2006). Analysis of the patient-therapist relationship in dynamic psychotherapy: An experimental study of transference interpretations. *The American Journal of Psychiatry*, *163*, 1739–1746.
Høglend, P., Bøgwald, K. P., Amlo, S., Marble, A., Ulberg, R., Sjaastad, M. C., Sorbye, O., Heyerdahl, O., & Johansson, P. (2008). Transference interpretations in dynamic psychotherapy: Do they really yield sustained effects? *American Journal of Psychiatry*, *165*, 763–771.
Høglend, P., Hersoug, A. G., Bøgwald, K. P., Amlo, S., Marble, A., Sørbye, O., Rossberg, J. I., Ulberg, R., Gabbard, G. O., & Crits-Christoph, P. (2011). Effects of transference work in the context of therapeutic alliance and quality of object relations. *Journal of Consulting and Clinical Psychology*, *79*, 697–706.
Horowitz, L. M., Rosenberg, S. E., Baer, B. A., Ureño, G., & Villaseñor, V. S. (1988). Inventory of interpersonal problems: Psychometric properties and clinical applications. *Journal of Consulting and Clinical Psychology*, *56*, 885–892.
Horvath, A. O., & Greenberg, L. S. (1989). Development and validation of the Working Alliance Inventory. *Journal of Counseling Psychology*, *36*, 223–233.
Kelley, H. H. (1950). The warm-cold variable in first impressions of persons. *Journal of Personality*, *18*, 431–443.
Kinley, J. L., & Reyno, S. M. (2013). Attachment style changes following intensive short-term group psychotherapy. *International Journal of Group Psychotherapy*, *63*, 53–75.
Klein, M. (1952). The origins of transference. *The International Journal of Psychoanalysis*, *33*, 433–438.
Levy, K., & Johnson, B. N. (2019). Attachment and Psychotherapy: Implications from Empirical Research. *Canadian Psychology/Psychologie canadienne*, *60*, 178–193.
Levy, K. N., Kivity, Y., Johnson, B. N., & Gooch, C. V. (2018). Adult attachment as a predictor and moderator of psychotherapy outcome: A meta-analysis. *Journal of Clinical Psychology*, *74*, 1996–2013.
Long, M., Verbeke, W., Ein-Dor, T., & Vrtička, P. (2020). *A functional Neuro-Anatomical Model of Human Attachment (NAMA):* Insights from first-and second-person social neuroscience. *Cortex*, *126*, 281–321. https://doi.org/10.1016/j.cortex.2020.01.010
Luborsky, L., & Crits-Christoph, P. (1998). *Understanding transference: The core conflictual relationship theme method*. American Psychological Association.
Magnani, R., Sabin, K., Saidel, T., & Heckathorn, D. (2005). Review of sampling hard-to-reach and hidden populations for HIV surveillance. *AIDS (London, England)*, *19*, 67–72.
Mallinckrodt, B. (2000). Attachment, social competencies, social support, and interpersonal process in psychotherapy. *Psychotherapy Research*, *10*, 239–266.
Mallinckrodt, B. (2010). The psychotherapy relationship as attachment: Evidence and implications. *Journal of Social and Personal Relationships*, *27*, 262–270.
Mallinckrodt, B., Gantt, D. L., & Coble, H. M. (1995). Attachment patterns in the psychotherapy relationship: Development of the Client Attachment to Therapist Scale. *Journal of Counseling Psychology*, *42*, 307–317.
Mallinckrodt, B., Porter, M. J., & Kivlighan, D. M. (2005). Client attachment to therapist, depth of in-session exploration, and object relations in brief psychotherapy. *Psychotherapy: Theory, Research, Practice, Training*. *42*, 85–100.
Mann, J. (1973). *Time-limited psychotherapy*. Harvard U. Press.
Meyer, D. E., & Schvaneveldt, R. W. (1971). Facilitation in recognizing pairs of words: Evidence of a dependence between retrieval operations. *Journal of Experimental Psychology*, *90*, 227–234.
Mikulincer, M. (1998). Adult attachment style and affect regulation: Strategic variations in self-appraisals. *Journal of Personality and Social Psychology*, *75*, 420–435.
Mikulincer, M., & Shaver, P. R. (2007). *Adult attachment: Structure, dynamics and change*. The Guilford University Press.
Mikulincer, M., & Shaver, P. R. (2016). *Attachment in adulthood: Structure, dynamics, and change* (2nd ed.). Guilford Press.
Mikulincer, M., & Shaver, P. R. (2018). An attachment perspective on family relations. In B. H. Fiese, M. Celano, K. Deater-Deckard, E. N. Jouriles, & M. A. Whisman (Eds.), *APA handbook of contemporary family psychology* (pp. 109–125). American Psychological Association.
Mikulincer, M., Shaver, P. R., & Berant, E. (2013). An attachment perspective on therapeutic processes and outcomes. *Journal of Personality*, *81*, 606–616.
O'Connor, S., Kivlighan, D. M., Jr., Hill, C. E., & Gelso, C. J. (2019). Therapist–client agreement about their working alliance: Associations with attachment styles. *Journal of Counseling Psychology*, *66*, 83–93.
Ogunnaike, O., Dunham, Y., & Banaji, M. R. (2010). The language of implicit preferences. *Journal of Experimental Social Psychology*, *46*, 999–1003. https://doi.org/10.1016/j.jesp.2010.07.006
Payne, B. K., Cheng, C. M., Govorun, O., & Stewart, B. D. (2005). An inkblot for attitudes: Affect misattribution as implicit measurement. *Journal of Personality and Social Psychology*, *89*, 277–293. https://doi.org/10.1037/0022-3514.89.3.277

This document is copyrighted by the American Psychological Association or one of its allied publishers.
This article is intended solely for the personal use of the individual user and is not to be disseminated broadly.
All rights, including for text and data mining, AI training, and similar technologies, are reserved.

Petrowski, K., Schmalbach, B., Schurig, S., Imhoff, R., Banse, R., & Strauss, B. (2019a). Implicit attachment schemas and therapy outcome for panic disorder treated with manualized confrontation therapy. *Psychopathology*, *52*, 184–190.

Petrowski, K., Schurig, S., Kirchmann, H., Singh, S., Banse, R., Imhoff, R., & Strauss, B. (2019b). Incongruence between implicit attachment schemes and unconscious attachment representations. *Journal of Nervous and Mental Disease*, *207*, 423–428.

Pollak, E., Wiegand-Grefe, S., & Höger, D. (2008). The Bielefeld attachment questionnaires: Overview and empirical results of an alternative approach to assess attachment. *Psychotherapy Research*, *18*, 179–190.

Puschner, B., Bauer, S., Horowitz, L. M., & Kordy, H. (2005). The relationship between interpersonal problems and the helping alliance. *Journal of Clinical Psychology*, *61*, 415–429.

Romano, V., Fitzpatrick, M., & Janzen, J. (2008). The secure-base hypothesis: Global attachment, attachment to counselor, and session exploration in psychotherapy. *Journal of Counseling Psychology*, *55*, 495–504.

Rosenzweig, S. (1936). Some implicit common factors in diverse methods of psychotherapy. *American Journal of Orthopsychiatry*, *6*, 412–415.

Sauer, E., Anderson, M. Z., Gormley, B., Richmond, C. J., & Preaco, L. (2010). Client attachment orientations, working alliances, and responses to therapy: A psychology training clinic study. *Psychotherapy Research*, *20*, 702–711.

Saunders, R., Jacobvitz, D., Zaccagnino, M., Beverung, L. M., & Hazen, N. (2011). Pathways to earned-security: The role of alternative support figures. *Attachment & Human Development*, *13*, 403–420.

Secord, P. F. (1958). Facial features and inference processes in interpersonal perception. In R. Tagiuri & L. Petrullo (Eds.), *Person Perception and Interpersonal Behavior* (pp. 300–315). Stanford University Press.

Shaver, P. R., & Mikulincer, M. (2002). Attachment-related psychodynamics. *Attachment & Human Development*, *4*, 133–161.

Simpson, J. A., Rholes, W. S., Oriña, M. M., & Grich, J. (2002). Working models of attachment, support giving, and support seeking in a stressful situation. *Personality and Social Psychology Bulletin*, *28*, 598–608.

Spangler, G., & Grossman, K. E. (1993). Biobehavioral organization in securely and insecurely attached infants. *Child Development*, *64*, 1439–1450.

Strauss, B., Kirchmann, H., Eckert, J., Lobo-Drost, A., Marquet, A., Papenhausen, R., Mosheim, R., Biebl, W., Liebler A., Seidler, K., Schreiber-Willnow, K., Mattke, D., Mestel, R., Daudert, E., Nickel, R., Schauenburg, H., & Höger, D. (2006). Attachment characteristics and treatment outcome following inpatient psychotherapy: Results of a multisite study. *Psychotherapy Research*, *16*, 579–594.

Strauss, B., Koranyi, S., Altmann, U., Nolte, T., Beutel, M. E., Wiltink, J., Herpertz, S., Hiller, W., Hoyer, J., Joraschky, P., Nolting, B., Stangier, U., Willutzki, U., Salzer, S., Leibing, E., Leichsenring, F., & Kirchmann, H. (2017). Partner-Related attachment as a moderator of outcome in patients with social anxiety disorder—A comparison between short-term cognitive–behavioral and psychodynamic therapy. *Psychotherapy*, *54*, 339–350.

Strauss, B. M. (2000). Attachment theory and psychotherapy research-Editor's introduction to a special section. *Psychotherapy Research*, *10*, 381–389.

Strauss, B. M., & Petrowski, K. (2017). The role of the therapist's attachment in the process and outcome of psychotherapy. In L. G. Castonguay & C. E. Hill (Eds.), *How and why are some therapists better than others?: Understanding therapist effects* (pp. 117–138). American Psychological Association.

Strauß, B., Altmann, U., Manes, S., Tholl, A., Koranyi, S., Nolte, T., Beutel, M., Wiltink, J., Herpertz, S., Hiller, W., Hoyer, J., Joraschky, P., Nolting, B., Ritter, V., Stangler, U., Willutzki, U., Salzer, S., Leibing, E., Leichsenring, F., & Kirchmann, H. (2018). Changes of attachment characteristics during psychotherapy of patients with social anxiety disorder: Results from the SOPHO-Net trial. *PLOS ONE*, *13*, Article e0192802.

Travis, L. A., Binder, J. L., Bliwise, N. G., & Horne-Moyer, H. L. (2001). Changes in clients' attachment styles over the course of time-limited dynamic psychotherapy. *Psychotherapy: Theory, Research, Practice, Training*, *38*, 149–159.

Waters, E., Crowell, J., Elliott, M., Corcoran, D., & Treboux, D. (2002). Bowlby's secure base theory and the social/personality psychology of attachment styles: Work (s) in progress. *Attachment & Human Development*, *4*, 230–242.

Winnicott, D. W. (1956). On transference. *The International Journal of Psychoanalysis*, *37*, 386–388.

Zelenko, M., Kraemer, H., Huffman, L., Gschwendt, M., Pageler, N., & Steiner, H. (2005). Heart rate correlates of attachment status in young mothers and their infants. *Journal of the American Academy of Child & Adolescent Psychiatry*, *44*, 470–476.

Zilcha-Mano, S. (2017). Is the alliance really therapeutic? Revisiting this question in light of recent methodological advances. *American Psychologist*, *72*, 311–325.

Zilcha-Mano, S., Dolev, T., Leibovich, L., & Barber, J. P. (2018). Identifying the most suitable treatment for depression based on patients' attachment: Study protocol for a randomized controlled trial of supportive-expressive vs. supportive treatments. *BMC Psychiatry*, *18*(1), Article 362.

Zilcha-Mano, S., Keefe, J., Chui, H., Rubin, A., Barrett, M. S., & Barber, J. P. (2016). Dropout in treatment for depression: Translating research on prediction into individualized treatment recommendations. *The Journal of Clinical Psychiatry*, *77*, 1584–1590.

Zilcha-Mano, S., McCarthy, K. S., Dinger, U., & Barber, J. P. (2014). To what extent is alliance affected by transference? An empirical exploration. *Psychotherapy*, *51*, 424–433.

Zilcha-Mano, S., Mikulincer, M., & Shaver, P. R. (2011). An attachment perspective on human–pet relationships: Conceptualization and assessment of pet attachment orientations. *Journal of Research in Personality*, *45*, 345–357.

Zilcha-Mano, S., Roose, S. P., Brown, P. J., & Rutherford, B. R. (2019). Not just nonspecific factors: The roles of alliance and expectancy in treatment, and their neurobiological underpinnings. *Frontiers in Behavioral Neuroscience*, *12*, 293–307.

Received May 16, 2020
Revision received February 15, 2021
Accepted February 21, 2021 ■

This document is copyrighted by the American Psychological Association or one of its allied publishers.
This article is intended solely for the personal use of the individual user and is not to be disseminated broadly.
All rights, including for text and data mining, AI training, and similar technologies, are reserved.